

Maturity falls away from the main campus toward Harbor

	CAPABILITY	MAIN CAMPUS	LAKESIDE	EASTGATE	RIDGEVIEW	ST. ANNE'S	HARBOR
PATIENT FLOW	Patient access	3	2	2	1	3	1
	Scheduling	3	3	2	2	2	1
	Revenue cycle	4	3	3	2	3	2
CARE DELIVERY	Clinical documentation	4	3	3	3	2	2
	Care coordination	2	2	1	1	2	0
	Supply chain	3	2	2	1	1	1
ENTERPRISE	Workforce management	2	2	2	1	1	1
	Data and analytics	3	2	1	1	2	0
	Cybersecurity	3	3	2	2	2	1

Absent
 Ad hoc
 Defined
 Managed
 Optimized

Source: Northfield Health System assessment, Sep 2026.

The front door trails every other group

	CAPABILITY	MAIN CAMPUS	LAKESIDE	EASTGATE	RIDGEVIEW	ST. ANNE'S	HARBOR	AVERAGE
FRONT DOOR	Patient access	3	2	2	1	3	1	2.0
	Scheduling	3	2	2	2	2	1	2.0
	Referral management	2	2	1	1	2	0	1.3
REVENUE	Revenue cycle	4	3	3	3	2	2	2.8
	Patient billing	3	3	2	2	2	1	2.2
	Denials management	3	2	2	1	2	1	1.8
DELIVERY	Clinical documentation	4	3	3	2	3	1	2.7
	Care coordination	2	2	1	1	2	0	1.3
	Supply chain	3	3	2	2	2	2	2.3
ENTERPRISE	Workforce management	3	2	2	2	1	1	1.8
	Data and analytics	3	2	1	1	2	1	1.7
	Cybersecurity	4	3	3	3	3	2	3.0

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Source: Northfield Health System assessment, Sep 2026.

CAPABILITY ASSESSMENT

The front door is the weakest group

- **Strong core.** Revenue rows reach level 3 or better at the main campus.
- **Thin edges.** Harbor sits at level 1 or below in most front-door and care rows.

WEAKEST GROUP

1.7

is the four-site average for the front door, the lowest of the four groups.

	CAPABILITY	MAIN	LAKE SIDE	RIDGEVIEW	HARBOR
FRONT DOOR	Patient access	3	2	1	1
	Scheduling	3	2	2	1
	Referral management	2	2	1	0
REVENUE	Patient billing	3	3	2	1
	Revenue cycle	4	3	3	2
	Denials management	3	2	1	1
CARE DELIVERY	Clinical documentation	4	3	2	1
	Care coordination	2	2	1	0
	Supply chain	3	3	2	2
	Quality reporting	3	2	1	0
	Population health	2	2	1	0
ENTERPRISE	Workforce management	3	2	2	1
	Data and analytics	3	2	1	1
	Cybersecurity	4	3	3	2

■ Absent
■ Ad hoc
■ Defined
■ Managed
■ Optimized

Source: Northfield Health System assessment, Sep 2026.

Front door scores well on process, poorly on data

CAPABILITY	PROCESS	TECHNOLOGY	DATA	PEOPLE	GOVERNANCE	WHAT WE FOUND
Patient access Front door	3	2	2	3	2	- Online booking covers 40% of visits - Two registration workflows by site - Add a single eligibility check
Scheduling Front door	3	3	2	3	3	- Templates differ by clinic - Self-scheduling live at three sites - Standardize visit-type rules
Referral management Front door	2	1	1	2	2	- Referrals arrive by fax and portal - No shared status view - Stand up a referral queue
Revenue cycle Revenue	4	5	3	3	4	- Rules engine at posting is mature - Close takes eight days, not five - Month-end review is manual

1 Ad hoc 2 Repeatable 3 Defined 4 Managed 5 Optimized

Source: Sample ratings for layout only; replace before use.

Revenue and care delivery: strong cores, weak handoffs

CAPABILITY HEAT MAP WITH NOTES (CONTINUED, 2 OF 3)

CAPABILITY	PROCESS	TECHNOLOGY	DATA	PEOPLE	GOVERNANCE	WHAT WE FOUND
Patient billing Revenue	3	3	3	2	3	- Statements are clear and on time - Payment plans sit outside the platform
Denials management Revenue	2	2	2	2	1	- Denial reasons are not coded - Rework is owned by no one
Clinical documentation Care delivery	4	4	3	4	3	- Templates are strong and reused - Voice capture is live in two units
Care coordination Care delivery	2	2	1	3	2	- Handoffs run on phone and email - Care plans live in three tools
Supply chain Operations	3	2	2	2	3	- Contracts are tracked by site - Item master has duplicates
Workforce management Operations	2	3	2	2	3	- Rosters are built by hand - Credentialing is on time

1 Ad hoc
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5 Optimized

Source: Sample ratings for layout only; replace before use.

Referrals, denials and care coordination score lowest

	CAPABILITY	PROCESS	TECHNOLOGY	DATA	PEOPLE	GOVERNANCE	WHAT WE FOUND
FRONT DOOR	Patient access	3	2	2	3	2	- Two registration flows across sites - Eligibility is checked after arrival
	Referral management	2	1	1	2	2	- Referrals arrive by fax and portal - No shared status view
REVENUE	Revenue cycle	4	5	3	3	4	- Rules engine at posting is mature - Month-end review is still manual
	Denials management	2	2	2	2	1	- Denial reasons are not coded - Rework has no owner
CARE DELIVERY	Clinical documentation	4	4	3	4	3	- Templates are strong and reused - Voice capture live in two units
	Care coordination	2	2	1	3	2	- Handoffs run on phone and email - Care plans live in three tools

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Start with denials and referrals, then lift the rest

	CAPABILITY	PROCESS	TECHNOLOGY	DATA	PEOPLE	GOVERNANCE	HORIZON	WHAT WE FOUND
FRONT DOOR	Patient access	3	2	2	3	2	NEXT	- Two registration flows across sites - Eligibility is checked after arrival
	Referral management	2	1	1	2	2	NOW	- Referrals arrive by fax and portal - No shared status view
REVENUE	Revenue cycle	4	5	3	3	4	LATER	- Rules engine at posting is mature - Month-end review is still manual
	Denials management	2	2	2	2	1	NOW	- Denial reasons are not coded - Rework has no owner
CARE DELIVERY	Clinical documentation	4	4	3	4	3	LATER	- Templates are strong and reused - Voice capture live in two units
	Care coordination	2	2	1	3	2	NEXT	- Handoffs run on phone and email - Care plans live in three tools

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Now this quarter, Next after, Later backlog

Source: Sample ratings for layout only; replace before use.

CAPABILITY	PROC	TECH	DATA	PPL	GOV	NOTES
Patient access	3	2	2	3	2	- Two registration flows - Eligibility checked late
Scheduling	3	3	2	3	3	- Rules differ by clinic - Self-scheduling at 3 sites
Referral management	2	1	1	2	2	- Faxes and portal mixed - No shared status view
Revenue cycle	4	5	3	3	4	- Rules engine is mature - Month-end review is manual
Patient billing	3	3	3	2	3	- Statements are on time - Plans sit outside billing
Denials management	2	2	2	2	1	- Denials are not coded - Rework has no owner
Clinical documentation	4	4	3	4	3	- Templates well reused - Voice capture in 2 units

CAPABILITY ASSESSMENT

Seven capabilities and the reason behind each score

- **Process leads.** It is the top score in five of seven rows.
- **Data lags.** Referral management scores 1 on technology and data.
- **Floor.** Denials has a 1 on governance.

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Source: Sample ratings for layout only; replace before use.

Enterprise capabilities trail on data and people

- **Security holds.** Governance reaches 5 in cybersecurity.
- **Data is the gap.** Master data has no owner in three of four rows.
- **People next.** Workforce and analytics both score 2.

1 Ad hoc

2 Repeatable

3 Defined

4 Managed

5 Optimized

CAPABILITY HEAT MAP WITH NOTES (CONTINUED, 3 OF 3)

CAPABILITY	PROC	TECH	DATA	PPL	GOV	NOTES
Supply chain	3	2	2	2	3	. Item master has duplicates . Contracts tracked by site . Clean item master first . Pool volumes across sites
Workforce management	2	3	2	2	3	. Rosters are built by hand . Credentialing is on time . Open reqs data is stale . Automate roster build
Data and analytics	2	3	2	2	2	. Reports rebuilt per team . Master data has no owner . Name data stewards . Publish a metric catalog
Cybersecurity	3	4	3	3	5	. Controls tested yearly . Access review quarterly . Vendor risk is tracked . Add live monitoring

Source: Sample ratings for layout only; replace before use.

Four capabilities need attention first

CAPABILITY	PROCESS	TECHNOLOGY	DATA	PEOPLE	GOVERNANCE	PRIORITY	WHAT WE FOUND
Referral management Front door	2	1	1	2	2	P1	- Referrals arrive by fax and portal - No shared status view
Denials management Revenue	2	2	2	2	1	P1	- Denial reasons are not coded - Rework has no owner
Care coordination Care delivery	2	2	1	3	2	P2	- Handoffs run on phone and email - Care plans live in three tools
Supply chain Care delivery	3	2	2	2	3	P3	- Item master has duplicates - Contracts tracked by site, not system
Revenue cycle Revenue	4	5	3	3	4	P4	- Rules engine at posting is mature - Month-end review is still manual

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P1 fix now, P2 next, P3 plan, P4 hold

Source: Sample ratings for layout only; replace before use.

CARE DELIVERY DETAIL

Care delivery and operations: one card per row

LOCATOR

A1					
A2					
A3					
A4					
A5					
A6					
A7					
A8					
A9					

1 Ad hoc 5 Optimized

Source: Sample ratings for layout only; replace before use.

A7 Clinical documentation

PRO	TEC	DAT	PPL	GOV
4	4	3	4	3

- Templates are strong and reused
- Voice capture live in two units
- Scale ambient notes to eight units

A8 Care coordination

PRO	TEC	DAT	PPL	GOV
2	2	1	3	2

- Handoffs run on phone and email
- Care plans live in three tools
- Pilot a shared care plan in oncology

A9 Supply chain

PRO	TEC	DAT	PPL	GOV
3	2	2	2	3

- Item master has duplicates
- Contracts tracked by site, not system
- Clean the item master first

Denials management is the weakest of the revenue rows

A6

Denials management

PRO	TEC	DAT	PPL	GOV
2	2	2	2	1

Overall 2 of 5

WHAT DRIVES THE SCORE

- Denial reasons are not coded
- Rework has no owner
- Appeals follow no playbook

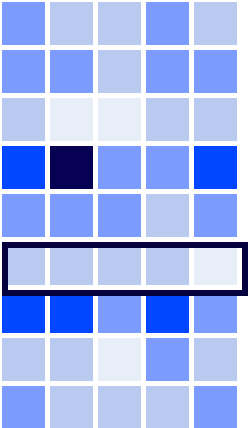
WHAT WE'D DO

- Code every denial at source
- Name one denials owner
- Write the appeal playbook

ACROSS A4 TO A6 (CONTINUED, 2 OF 2)

- **Process outruns data.** Revenue cycle scores 4 on process but 3 on data.
- **Denials are the gap.** A6 is the only revenue row with a 1.
- **Lowest cell.** Governance in denials management scores 1.
- **Sequence.** Code denials first; rules and billing changes follow.

LOCATOR



ROW A6

1 Ad hoc 2 Repeatable 3 Defined 4 Managed 5 Optimized

Source: Sample ratings for layout only; replace before use.

Revenue cycle and billing: process is ahead of the platform

A4

Revenue cycle

PRO	TEC	DAT	PPL	GOV
4	5	3	3	4

Overall 4 of 5

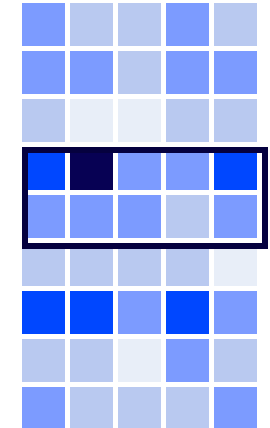
WHAT DRIVES THE SCORE

- Rules engine at posting is mature
- Month-end review is still manual
- Close takes eight days, not five

WHAT WE'D DO

- Extend rules to the remaining sites
- Automate the day-one review
- Publish a daily close status

LOCATOR



ROWS A4-A5

A5

Patient billing

PRO	TEC	DAT	PPL	GOV
3	3	3	2	3

Overall 3 of 5

WHAT DRIVES THE SCORE

- Statements are clear and on time
- Payment plans sit outside the platform
- Lockbox posting runs on a spreadsheet

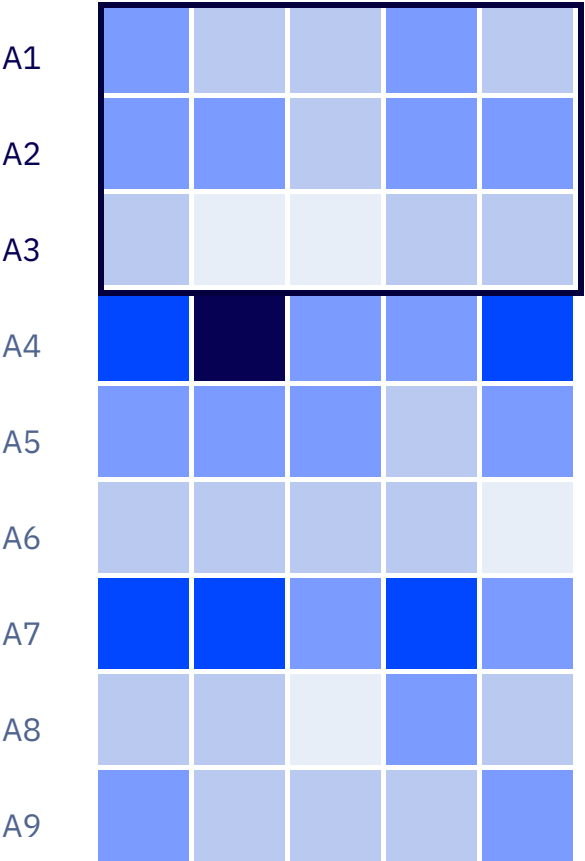
WHAT WE'D DO

- Move plans into the billing system
- Retire the lockbox spreadsheet
- Add text-to-pay for balances

1 Ad hoc 2 Repeatable 3 Defined 4 Managed 5 Optimized

Source: Sample ratings for layout only; replace before use.

LOCATOR



1 Ad hoc 5 Optimized

FRONT-DOOR DETAIL

Front-door rows in depth

A1 Patient access

PRO	TEC	DAT	PPL	GOV
3	2	2	3	2

- **Drives it.** Two registration flows across sites; eligibility checked after arrival.
- **We'd do.** Run one eligibility check at booking; train front desks on a single script.

A2 Scheduling

PRO	TEC	DAT	PPL	GOV
3	3	2	3	3

- **Drives it.** Self-scheduling live at three sites; visit-type rules differ by clinic.
- **We'd do.** Standardize visit-type rules; automate the waitlist.

A3 Referral management

PRO	TEC	DAT	PPL	GOV
2	1	1	2	2

- **Drives it.** Referrals arrive by fax and portal; no shared status view.
- **We'd do.** Stand up one referral queue; track leakage by specialty.

Source: Sample ratings for layout only; replace before use.

Revenue cycle is the group to look at first

CAPABILITY	PROCESS	TECHNOLOGY	DATA	PEOPLE	GOVERNANCE	OVERALL
A1 Patient access	3	2	2	3	2	2
A2 Scheduling	3	3	2	3	3	3
A3 Referral management	2	1	1	2	2	2
A4 Revenue cycle	4	5	3	3	4	4
A5 Patient billing	3	3	3	2	3	3
A6 Denials management	2	2	2	2	1	2
A7 Clinical documentation	4	4	3	4	3	4
A8 Care coordination	2	2	1	3	2	2
A9 Supply chain	3	2	2	2	3	2

1 Ad hoc 2 Repeatable 3 Defined 4 Managed 5 Optimized

Source: Sample ratings for layout only; replace before use.

- **Reference badges.** Each row keeps its code on the slides that follow.
- **Overall.** The rounded mean of five dimensions.

NEXT TWO SLIDES

Detailed notes follow for A4–A6

A4 Revenue cycle

A5 Patient billing

A6 Denials management

Revenue cycle is the group to look at first

	CAPABILITY	PROCESS	TECHNOLOGY	DATA	PEOPLE	GOVERNANCE	OVERALL
FRONT DOOR	A1 Patient access	3	2	2	3	2	2
	A2 Scheduling	3	3	2	3	3	3
	A3 Referral management	2	1	1	2	2	2
REVENUE	A4 Revenue cycle	4	5	3	3	4	4
	A5 Patient billing	3	3	3	2	3	3
	A6 Denials management	2	2	2	2	1	2
CARE DELIVERY	A7 Clinical documentation	4	4	3	4	3	4
	A8 Care coordination	2	2	1	3	2	2
	A9 Supply chain	3	2	2	2	3	2

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A4 Revenue cycle

A5 Patient billing

A6 Denials management